

HPV Vaccination and Anogenital Warts

- Smoking is associated with persistence of anogenital HPV infection. Although there is no evidence that smoking cessation improves wart treatment outcomes, there is a clear individual and public health rationale for advising smoking cessation. (1C)
- **Gardasil®** and **Gardasil9®** (both by MSD Vaccines) provide durable protection against HPV genotypes 6 and 11, which cause the majority of anogenital warts.
- A recent trial suggested a possible benefit of using the vaccine in combination with topical **imiquimod** or **podophyllotoxin** for treatment or prevention of recurrence of genital warts, but this remains unconfirmed.
- There is no evidence of benefit from vaccination in individuals with HPV infection but no clinical disease.
- Vaccination prior to sexual debut maximizes protective benefits.
- HPV vaccination strategies vary by country.
- Substantial reductions in genital wart incidence have been observed in young women and heterosexual men in **Australia** following the introduction of **Gardasil®** vaccination in schoolgirls.
- A reduction in genital warts was also observed in women and heterosexual men aged 15–19 in the **UK** following the introduction of the **bivalent vaccine Cervarix®** (GlaxoSmithKline), despite its lack of coverage for HPV 6 and 11, suggesting possible cross-protection or herd effects.